

Joint Replacement Surgery for Arthritis — A Complete UK Guide

Everything you need to know about knee and hip replacement surgery on the NHS: when it's right, what to expect, and how to prepare and recover.

By **Living With Arthritis Editorial Team** · Pending clinical review ·
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Joint replacement surgery sounds daunting. It is natural to feel anxious — but it is also one of the most successful operations in modern medicine. In the UK, more than 200,000 hip and knee replacements are performed each year, and over 90% of people report significant improvement in pain and mobility.

This guide explains everything in plain English, from deciding if surgery is right for you to what recovery actually feels like.

When Is Joint Replacement the Right Choice?

Joint replacement is not the first option. You and your doctor will usually consider it when you have severe persistent pain affecting your sleep and daily activities, other treatments have not provided enough relief, X-rays show significant joint damage, and your quality of life is being seriously affected. It is a decision you make *with* your orthopaedic surgeon, not one they make *for* you.

Knee Replacement Surgery — What to Expect



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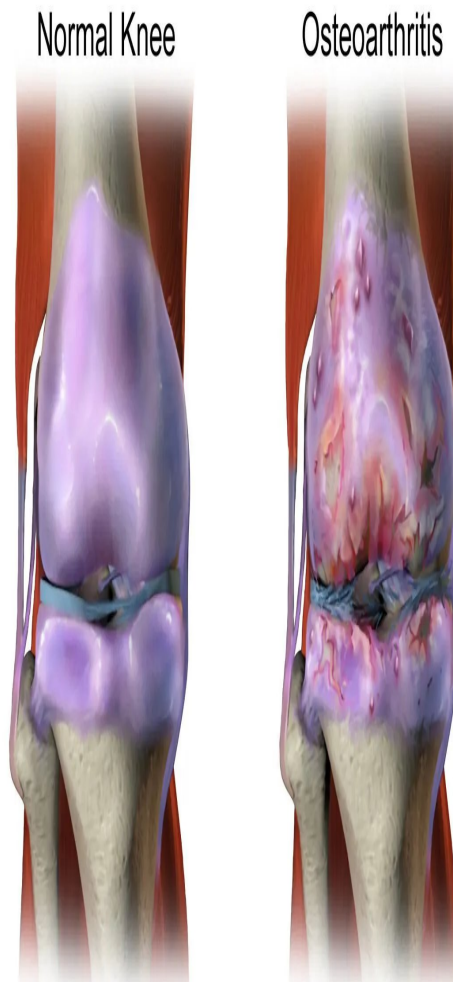
A total knee replacement (knee arthroplasty) involves removing the damaged surfaces of the knee joint and replacing them with metal and plastic implants. The operation takes about 1–2 hours, usually under spinal anaesthetic or general anaesthetic. You will typically stay in hospital for 1–3 days. You will be encouraged to stand and take a few steps with a walking frame within 24 hours, and a physiotherapist will guide you through the first exercises. Most people notice improved mobility within 6–12 weeks, with continued gains for up to a year.

Hip Replacement Surgery — Recovery and Results

Hip replacements follow a similar pattern to knee replacements. The damaged ball-and-socket joint is replaced with artificial components. Hip replacement generally has a slightly quicker early recovery than knee replacement. Many people go home within 1–2 days. Use crutches or a walking frame for 2–6 weeks, follow hip precautions if advised, and most people can drive after about 6 weeks (check with your insurance provider). You may return to work within 6–12

weeks depending on your job.

Getting on the NHS Waiting List



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The pathway typically involves GP referral to an orthopaedic consultant, specialist assessment with X-ray review, decision to proceed, pre-operative assessment (blood tests, heart tracing, health checks), and finally a surgery date. The NHS Constitution target is a maximum of 18 weeks from referral to treatment, but waits may be longer in some areas.

Preparing for Surgery (Pre-habilitation)

What you do before surgery significantly affects how well you recover after. Strengthen your muscles around the joint — stronger muscles mean faster recovery. Maintain a healthy weight

— excess weight puts extra stress on your new joint and increases surgical risks. Stop smoking — it slows healing and raises complication risks. Eat well, and plan your home: arrange help with shopping, cooking, and transport for the first few weeks, and clear trip hazards.

Recovery Timeline and Rehabilitation



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Everyone recovers at their own pace. Week 1–2: walking with a frame or crutches inside the house, daily physio exercises, swelling and discomfort peak then improve. Weeks 2–6: gradually reduce walking aids, start walking outside, continue exercises. Weeks 6–12: walking freely, return to driving, may return to office-based work. 3–6 months: significant improvement in pain and function. 6–12 months: continued gains, resume low-impact sports. Knee replacements tend to have a slightly slower recovery than hips.

Life After Joint Replacement

The goal is a life with less pain and more movement. Most people can walk without pain, sleep through the night, return to hobbies like gardening and swimming, and travel comfortably. You should avoid high-impact activities like running or jumping. Modern implants often last 15–25 years or more.

Alternatives to Surgery

Surgery is not the only way forward. Some people manage well with weight loss (even 5–10% of body weight can significantly reduce knee pain), a tailored physiotherapy exercise programme, a pain management programme combining physical and psychological strategies, walking aids and supports, or steroid injections for temporary relief. Always explore these with your GP or physiotherapist first.

Q&A

Question: When Is Joint Replacement the Right Choice?

Short answer: Joint replacement is not the first option. You and your doctor will usually consider it when you have severe persistent pain affecting your sleep and daily activities, other treatments have not provided enough relief, X-rays show significant joint damage, and your quality of life is being seriously affected. It is a decision you make with your orthopaedic surgeon, not one they make for you.

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Arthritis UK: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.